

Benchmark Course Synthesis Paper

Raymond Wagoner

School of Behavioral Sciences, Liberty University

Author Note

Raymond Wagoner

I have no known conflict of interest to disclose.

Correspondence concerning this article should be addressed to Raymond Wagoner. Email:

rdwagoner1@liberty.edu

Course Synthesis Paper

From an integrationist perspective, the Jones family (See Appendix A for a complete case study) struggles to make sense of suffering and adversity on two fronts: theological and psychological. On the theological front, they are faced with questions about God's character and sovereignty, including His goodness, love, care, ability, and desire to heal (Grudem, 2022). On the psychological front, they are faced with emotions, distress, and safety. The overlap happens when they start asking what God's sovereignty has to do with their psychology and what psychology can offer them in their search for truth (Entwistle, 2021). The Jones family comes to me with a head start on the ability to attach to one another securely when I conceptualize them through the lens of attachment theory. It will not be difficult to help them find relief from their distress with one another and with their extended family and other support systems. Through an integrationist lens, it will not be difficult to use this secure attachment to help them find a secure attachment filter with God (Entwistle, 2021; Hall, 2022). As this family learns to submit their psyche to the sovereignty of God and to wrestle with the human limitations of their theological Weltanschauung, we will work together to lead them into a more fully aware relationship with God (Entwistle, 2021). With this set of glasses on, we will utilize attachment theory and neuropsychological interventions to prime each member to secure attachments with the other and with God. After the initial intake of informed consent and setting expectations, I asked each person to self-report on the Attachment-style Questionnaire. Once completed, we scheduled sessions with each member individually, Mr. and Mrs. Jones together, and the family together in one session. Let us start this conceptualized treatment plan with one member of the family at a time.

Joe Jones

I will start with Joe, Dad, not because of any philosophy of marriage or family or theology of roles in either of those concepts, but because I get the sense he might be the most complex of the clients next to Jessica. He is complex because he is struggling with his ability to lead a family through a crisis; his protective instincts for his wife and children are coming online, but his avoidant attachment to threats out of his control is currently winning. He desires to know God well enough through his study of theology that he can stay faithful and grounded (although he does not know what emotional regulation is yet nor how to become aware of his dysregulation and disintegration). He leads people through these crises all the time in his role as a pastor, but he has never experienced this level of discomfort from an acute crisis himself. He is internally struggling with his theology of God's character and sovereignty but does not want to let on to the family (especially Jessica) that he is questioning what he has called and taught as truth for many years.

My plan with Joe is to create a space for him that will allow him to cry when he needs to and allow him to bring questions about God and human existence out into the open when he needs to. There are spaces that allow us as humans to sense God's presence more acutely than other spaces (Entwistle, 2021). These spaces are not some mystical doors to the other side that we must find. These spaces are the places we create to perceive God as we slow our hearts and minds to connect with the Holy Spirit intentionally. I am introducing this idea here, but I will use this awareness with all the members individually and together. As well as a space, I will offer Joe (and every member of the family) a secure base for regulation and integration (Hall, 2022) through my own grounded and attuned presence in the relationship. Offering this attunement will

allow for an effective therapeutic alliance, a sense of safety and security, and a sense of being seen and soothed for Joe (Hall, 2022).

This plan will allow me to teach Joe how to regulate his emotions through psychoeducation of the limbic system, the threat system, mindfulness, and secure attachments (Hall, 2022). It will also allow me to remain open to Joe's questions about God as a loving, kind, gracious, caring, healing, powerful, potent creator and sustainer (Grudem, 2022). I need not force Joe into any theological position because my secure relationship with God allows me to hear others' struggles without straining my faith (Entwistle, 2021; Grudem, 2022; Hall, 2022). The countertransference that could come up over questioning God's character can be relieved as I stay grounded and emotionally regulated by attuning intra and interpersonally (Hall, 2022). This relationship, where I offer Joe a secure base to return to as he struggles with feeling adrift due to his wife's diagnosis, is the main modality I will employ. Interventions such as mindfulness activities, priming to secure attachments, validation of Joe's emotions and questions, and empathy for his distress and current situation will help Joe remain steady and steadfast for his wife and children as they embark on a journey of the unknown. This will allow Joe to become a secure base for his children to return to as they struggle with the possibility of losing their mother and the strain of questioning where God is in this circumstance. Finally, this will allow Joe to sit with his wife during treatments and doctor visits and not stare off into space as his protective limbic system tries to help him avoid hearing hard things. But what about his wife Jessica? Will the same plan help her with her distress?

Jessica Jones

Yes and no. The same theory of change and human existence will help her, but not in the same ways. Jessica is not the avoidantly attached human her husband is. Her lived experiences

and view of God have created a preoccupied attachment filter for Jessica's threats (Hall, 2022). When Jessica perceives a threat to her existence or those whom she loves, she goes into action, trying to create safety for herself and anyone else she loves and feels responsible for. While this can be helpful to others by creating a relationship in which they are safe, seen, soothed, and secure (Hall, 2022), she is not able to pay attention to her own fears and questions about God. Her system automatically tells her that she cannot doubt or try to control the situation on her own. I sense when I meet with her for the first time individually that her preoccupied attachment is fearful to hear that God is displeased with her for some reason or that God is causing this cancer for His glory or her sanctification (Gruden, 2022). She reported that her biggest fear is that her death will mean she has disappointed Joe and the kids. She is not afraid to die and is confident in her salvation and eternity with God, but how will her family survive without her?

One step I will take with Jessica and Jim is to administer the God attachment inventory from Mayfield's *Attached to God* (2022, pp. 10-14). Both family members will benefit from the awareness this inventory will bring. I do not intend to tell them they are wrong in their view of God; I simply want to help them see that they can invite God in and not push him away. I sense Jessica has a shame-filled attachment to God that is not allowing her to partner with him in her distress and anxiety over abandoning her children. I sense this diagnosis is touching on a part of her theological paradigm that wonders if her sin is a constant agitation to God and that he is fed up with her not being obedient to the point of a last-ditch effort to get her to change her thoughts and lack of faith. This, coupled with her system's need to act, is putting her in a difficult and distressful dilemma.

As with Joe, creating a space where Jessica can be honest and transparent with God and me will set the stage for her to sense a secure base with us. However, this space will look

different from Joe's. She will not react by spacing out or changing the subject to the big game next weekend. She will react by offering solutions, asking for ways to increase her faith, and changing the subject from her pain and diagnosis by shifting to Joe and the kids. While deftly navigating the boundaries around what Joe and the kids are telling me and how they are doing, in my professional opinion, I do not mind giving her some thoughts on the scripture verses she brings into the sessions. I am careful not to use the scriptures to pivot to a counseling intervention (Entwistle, 2021). Rather, I sincerely want to substantiate her need for answers from her only source of truth and validate her need for God to come find her. As I corroborate her need to protect her children and not be a disappointment to her husband, I want to show her that God is that loving, kind, compassionate Father she hopes he is (Grudem, 2022). I want to assure her (and psychologically soothe her) that God will provide for her children should she die. By helping her find a secure base in God and in me, Jessica will be more able to connect with other people in her support system and hold a healthier view and relationship with God (Hall, 2022; Entwistle, 2021).

The interventions of validation, empathy, unconditional positive regard (by nonjudgmental responses to her scriptural questions), priming to attachment, and mindfulness will forge a path for Jessica to communicate and connect with her family while still attending to her own physical and emotional needs without feeling shame for being sick. As she endures treatments, being too tired to go to kids' games and concerts, and not physically available for Joe, Jessica can soothe herself with the images of Jesus helping the woman with the issue of blood she so peacefully connected with in the session.

Marriage Relationship

Speaking of not being physically available to Joe, our couple's session was focused mostly on how Joe's avoidant attachment and Jessica's preoccupied attachment were creating seemingly unresolvable conflict over the smallest things nowadays. After remarking that new ways of connecting and new patterns of transparency would be necessary during this season, the couple were less blaming of each other and more empathetic of each other's emotions. Joe did take the risk of telling Jessica that he was frustrated that she wasn't able to connect with him physically. This had always been a way for him to know she cared and a way to feel wanted. Jessica produced more scripture about the wife needing a sabbatical from sexual intimacy on occasion, but this did not soothe Joe. In an effort to integrate her theological position and Joe's emotional/psychological needs, I asked Jessica to relate back to Joe what she heard him say about the lack of connection. The theory behind this intervention is that she will see the situation from Joe's perspective. The plan with the marriage relationship is to help each of them perceive and receive the other's perspective in a non-threatening and validating manner. In keeping with this plan, as in the example above, sharing the science behind attachment theory and the results from the questionnaire with the two of them, combined with moderating a conversation, will theoretically bring empathy and compassion back into the relationship during this season. The fact that they have a secure attachment to each other means that we can look at times when they have been able to soothe each other emotionally and find ways to provide emotional intimacy and spiritual intimacy, even if physical intimacy is not possible for a time.

Once they have restored this secure attachment, the plan evolved into allowing each of them to convey what a diagnosis and treatment means to them theologically, relationally, and emotionally. Keeping them on track and helping them talk about hard things when their threat

systems want to distract them with avoidance or preoccupation means pausing to pray sometimes to invite God's wisdom, priming them back to attachment sometimes to remind them they are allies and not enemies, and working with those systems by validating the fears. Interventions that include setting up a dialogue and not simply asking individual questions will be an important modality in helping the two learn to have emotional and spiritual intimacy outside of the counseling session. Setting up scenarios where Joe is able to soothe Jessica during a chemo treatment, or Jessica is able to soothe Joe after Jana breaks something else will provide avenues for safety and security and a sense of being seen and soothed (Hall, 2022) outside of my presence. Concisely stated, the plan with the couple is to give them a space where they can touch God (Entwistle, 2021), pray to the one they know has answers (Grudem, 2022), and allow themselves to have emotion without fear of affecting the children in negative ways.

Jim Jones

Continuing with the children, Jim's self-blaming is concerning to me. His avoidant attachment to threats and shame-filled attachment to God (Hall, 2022; Mayfield, 2022) are causing him to react by turning inward. He relates to me that following the rules has always helped him succeed, whether it was in school or in scouts. His theological formulations have led him to believe the same about God: just do what I'm supposed to, and everything will be okay. After walking Jim through the self-administered God attachment inventory and the results from his Attachment Style Questionnaire, I spent some time reflecting on the biblical account of Jesus calming the storm from Mark Chapter 4. I was not surprised when Jim's main takeaway from the story was that the disciples did not have enough faith to calm the storm themselves. However, I asked him why the disciples thought no harm would come to them while Jesus was in the boat with them. His response was that Jesus does guarantee our safety, but after he thought about that

response for a minute, he realized that he was not seeing the full picture. Jim admitted that it was possible for Jesus to be in the boat with us in the storm and calm our hearts even if he did not calm the storm (Grudem, 2022). I recognized out loud that Jim's circumstances were definitely out of his control, and his avoidant attachment and his shame-filled attachment to God were distracting him from the logical processing he needed to see Jesus in the boat with him. He agreed that the chaos and pain were causing him to blame himself for his mom's diagnosis and the family's response to it (Grudem, 2022).

I continued to help Jim see that he indeed needed faith to feel unashamed and connected to God/Jesus but that his emotional regulation would help him see the truth in the chaos. Teaching him emotional regulation techniques like breathing, grounding, and Christian mindfulness followed these discussions and are part of my plan with Jim. The fullness of the plan includes challenging Jim's cognitive distortions as he believes he has somehow caused his circumstances (similar to the example above). It also includes interventions such as a narrative therapy style "restorying". However, the integrated approach says that we allow God to speak into Jim's identity as a faithful follower regardless of his circumstances (Grudem, 2022) and not simply ask him where he wants to take himself.

Jim is away at college and needs to return to his studies as soon as possible. Therefore, part of the plan with Jim needs to be to help him participate in the family session and then connect with a counselor closer to school. I have also discussed with Jim and the family the possibility of Jim participating in the family sessions virtually in the future. After ensuring Jim that the three references I gave him were reputable colleagues and would tend to his desire to consider his Christian worldview, I encouraged him to contact them on his own. His mother and father also talked to the pastor from the church Jim attends while in class and asked him to check

on Jim regularly and for any counselor recommendations from his local knowledge. As Jim left my office, I had the confidence that he would be okay as he continued to connect to God and others.

Jana Jones

Jana is a different story. Jana is angry. She does not want to talk to me about her mother's illness. She sullenly sits on the couch, seethes, and grinds her teeth as she watches the second hand on the clock. She is reacting similarly to her mother with a preoccupied attachment response, but her inability to make sense of the situation or make it go away is causing agitation and frustration. She is not able to soothe herself with scripture as her mother does, but she has the same questions about God's mercy and grace. I would like my plan to be that we navigate the results of her attachment questionnaire and work toward a measured response to God and others (Entwistle, 2021; Grudem, 2022). However, I sense my plan will be to build a therapeutic alliance with her over time. It won't happen overnight, but I trust the alliance will create a place for her to share her hardest thoughts and emotions. This alliance building looks like treating her like an adult. With the exception of safety concerns, I let her know that I would not discuss anything we talked about with her parents. I also gave her a copy of the informed consent and asked her to read it with me and sign it. Technically, her parents have given consent, but I wanted her to know that I was considering her as an individual. I also asked her what her goal with counseling was. I didn't get much of a response at first, just a "bombastic side-eye," as the interns call it. Nevertheless, she eventually softened enough to say she just wanted to help her mom. This simple comment gave me insight into her anger. She could not help her mom and was trying to let the world and God know it wasn't fair. I agreed out loud that sometimes life isn't fair (Grudem, 2022). I asked her questions about where her sense of equity comes from and how this

world is supposed to be (Entwistle, 2021). She answered some questions as the incredibly intelligent young person she is, but a lot of the questions seemed to fall on deaf ears. My hope is that she is considering all we talk about and all I encourage her towards and will talk about her emotions either in the individual sessions or the family session. For now, I encourage her to journal daily. She seemed to take well to that (simply noted by her eyes being directed downward and not in a side-eye glance at me).

The interventions with Jana are more subtle and more person-centered than attachment-based. I wonder if I can be a secure attachment base for her. I will validate her emotion, but it will be based on non-verbal communication, much like one would in child-centered play therapy. I will notice her anger as she glances at me with squinted eyes. I will notice her sadness as she exclaims that she just wants to help her mom. I will notice her lack of control as she plops down on the couch and slumps as though she is presenting for an inquisition.

After the first session, a brilliant idea came to me. Actually, Sarah, the intern, said, “I would have given anything to do counseling with my mom at that age,” as she watched her leave. Great idea, I thought to myself. Why not have Mom come in for just a session with the two of them? It may turn into me leaving and the two of them crying for an hour, but that would be better than Jana continuing to break stuff. Then, since Sarah brought it up, I decided to give her a shot at meeting with the two of them. I called Mom and suggested the idea, and she agreed immediately. Since Sarah, the intern, might have a better chance of connecting with and being secure for Jana, we will consider transitioning her to Sarah in the future. For now, I will meet with Jana and Jessica for at least one session so Jana can have some time to communicate with her alone. Hopefully, this will give her a healthy outlet for all of that emotion.

John Jones

John, the youngest Jones, is not like his siblings. He is not pensively silent like his sister nor respectfully agreeable like his brother. Once I had John alone in the session, he talked so much that I could hardly get my speech in. Most of his comments were directed at his family's beliefs. He is confused by what his family and church have always told him about God and how different that is from how they are all reacting to this news. If they really believed what they told me, God is in control and able to do great things, they would not be freaking out over this. His avoidant attachment and confusion over the incongruence are causing him to retreat into the only thing that is currently soothing him: video games (Hall, 2022). However, this perpetuates the cycle of shame as he remembers all his parents and youth pastor have told him about spending too much time on games. He is shamed for his shame by his own threat system (Hall, 2022). However, most of this shame is currently directed at his father, who is not engaging him in the questions he has about faith and healing (Entwistle, 2021; Grudem, 2022). Mom is more helpful than Dad, and she is sick, he says. He reported not understanding his father's willingness to pray with everyone else in the church but not his own family. John may be just as angry as his sister, but his avoidant use of video games is more acceptable than his sister's angry outbursts and violent aggression. However, there is more reason to believe that John will engage me in conversation, even if it is an antagonistic conversation about my own beliefs. I can handle this antagonism because of my internal attunement (Hall, 2022) and my ability to hold theological positions with an open hand (Entwistle, 2021).

The plan with John is to allow him to openly disagree with his family's dinner table rhetoric that simply ignores the real issue (his words). I will create the same safe space as I have with the other members of the Jones family so John can externalize his lack of control, lack of

being included, and lack of understanding about God. Giving him a safe space to confront God and his family (though implicitly and not explicitly) will create a sense of safety with me (Entwistle, 2021; Hall, 2022). I will employ validation, unconditional positive regard, empathy, supportive listening, and congruence as interventions with the youngest member of the family. All of these will give John an assurance that he can have the same conversations with Mom and Dad that he is vicariously having with them in session. Employing an empty chair technique along with the other interventions will help John formulate his emotional language and provide confidence in navigating the dinner table with kindness and vulnerability.

John is also a great candidate for our group of young men desiring to connect in healthy relationships rather than to isolate with games or other unhealthy behavior. Once a week, the group meets at the office and discusses what it was like to transition from quarantine to high school without much help. Some of these young men are using pornography or alcohol or sex to soothe the anxiety of emotional dysregulation. But, like John, many have seemingly innocuous behaviors that have the same effect on them. One of our male interns leads the group through process therapy in hopes of regulating them, helping the feelings of aloneness, and priming them back to the secure attachments of God and others (Entwistle, 2021; Grudem, 2022; Hall, 2022). After discussing the idea with Dad, John and Dad agree that this would be a good idea. Dad even asks if we have a group for him. We will work on that one. For now, Dad's group will have to be the family unit. I assure him that he can stay present and aware during those sessions.

Jones Family

Succinctly stated, the plan for the family as a whole is to create a safe space for each of them to be seen, soothed, safe, and secure (Hall, 2022) by removing psychological shame and theological expectations and modeling transparent connection (Entwistle, 2021; Grudem, 2022).

The family sessions will follow the principles of group process theory with the understanding that Mom and Dad have a distinct power differential. However, guiding the family through the principles of process therapy will help them engage each other on a level playing field in the session. This model will also give the family members the space to confront their fears, doubts, questions, pain, and insecurities with one another. Employing the principles of process theory, such as setting guidelines agreed upon by all members, will allow Jana and John specifically and all the members generally to participate on equal footing.

On one occasion, Mom brought up the account of the woman with the blood disease from Mark 5. Her hope was to show the family that Jesus was always aware of their circumstance. John tried to take the opportunity to discuss the theology of healing. Jana rolled her eyes because Mom brought this up the other day without giving her a chance just to be sad. Jim was ready to discuss the issue but checked in with Dad to see if he was going to participate or not. Dad tried to avoid the topic (understandably sensing his wife might not be with him much longer) by asking her how she was feeling. Looking at the dynamics in the room, Kara (the co-leader) and I moved the family into a discussion of the meaning of the passage. We tried to direct a conversation around how emotions impact our view of God (Entwistle, 2021; Hall, 2022; Grudem, 2022). We eventually led the family into a more specific discussion of how everyone in the room feels about Mom's cancer in view of God's ability to heal (Entwistle, 2021; Grudem, 2022). Using parallel processing and horizontal processing, we tied each person's comments to the other and asked questions that led to a deeper expression of emotion. The first session was understandably clunky, and we received more than one bombastic side-eye from more than just Jana. But, our ability to navigate the ambivalence of theology and doctrine with an understanding of the limits of human reasoning and use our understanding of psychology to guide interpersonal processing

won the day (Entwistle, 2021; Grudem, 2022). Eventually everyone told Mom why they loved her and how much she meant to them in front of the whole family (including Jana and Joe). Eventually, Jessica agreed to let others help her and to let Jana tell her when she was not receiving kindness from the people who just wanted to help. Eventually, everyone cried over God's love in the midst of the storm (including the leader and co-leader). As you can imagine, there was no way that every question, doubt, pain, and wound was going to be addressed. However, we created the ability to communicate with one another in a way that brought a sense of safety and security and allowed everyone to be seen and soothed in the family sessions. This ability will hopefully translate into sitting in chemo treatments, family dinners that feel way different, days when it feels like Mom isn't going to make it, and holidays when Mom is too tired to do the traditions. No one member is responsible for all of this and each member can add to the secure attachment filters they are capable of sensing.

Conclusion

Integration is the ability to cooperate with what we do not fully know. We do not fully know that our theology is correct. We do not fully know how to interpret the findings of psychology. We do have the assurance that God is actively involved in and presently coordinating how we read the Scriptures he has provided us and how we interact with and understand the natural world we live in (Entwistle, 2021). For the Jones family, this meant needing to rethink some doctrines of God's character, God's activity, God's sovereignty, and humanity (Grudem, 2022). It also meant having to come to terms with the purpose of emotion and emotional reactions. The plan for the family and individuals was not to fore a theological or psychological belief. The plan was not to separate theology from psychology by telling them they would have to go elsewhere to talk about God. The plan was not to separate psychology

from theology by telling them that everything they were experiencing was a product of sin and that they would need to pray more and read their bible more (Entwistle, 2021). The plan was to give them all a safe space to transparently interact with each other and God in a way that helped them sense safety, being seen, being soothed, and security to create purposeful and healing relationships (Entwistle, 2021; Grudem, 2022; Hall, 2022).

References

- Entwistle, D. (2021). *Integrative approaches to psychology and Christianity: An introduction to worldview issues, philosophical foundations, and models of integration* (4th ed.). Cascade Books.
- Grudem, W. (2022). *Bible doctrine: Essential teachings of the Christian faith* (2nd ed.). Zondervan Academic.
- Hall, T. (2022). *The connected life: The art and science of relational spirituality*. Inter-Varsity Press.
- Mayfield, K. (2022). *Attached to God: A practical guide to deeper spiritual experience*. Zondervan.

Appendix A

The Jones family (parents Joe and Jessica, and children Jim, Jana, and John) recently filled out all their paperwork to begin counseling with you at your private practice. You have had one intake session with the family and learned the following: Mr. Jones, (43 yo) is a bivocational pastor, and part-time computer programmer. Mrs. Jones (43 yo) is a real estate agent. Son Jim is 19 years old and a freshman in college, daughter Jana is 17 and a junior in high school, and son John is 16 and a sophomore in high school. The Jones family have always been close, and all have good feelings towards each other, and they are all sincere and committed Christians. Two weeks ago, after some symptoms, Mrs. Jones was diagnosed with stage 4 lung cancer and told that she has a 10% chance of survival, and likely has about 4-6 months to live. Mrs. Jones has a rare form of cancer that is inconsistent in how it responds to traditional cancer treatments. Mrs. Jones is set to begin various traditional and nontraditional intense cancer treatments, in hopes of beating the cancer and surviving. Both sets of grandparents as well as some aunts and uncles live nearby and are offering to help in any way they can, as is the church where Mr. Jones is the pastor.

During the intake you notice that many of the family members are reacting differently to the medical news. Mr. Jones stares at the wall much of the session, periodically says repeatedly- "This just doesn't make sense...she was fine." At one point Mr. Jones says, "I want to be able to say- 'The Lord gives, the Lord takes away, blessed...'" (and then he chokes up and can't finish the sentence). Mrs. Jones is very confident and keeps shifting her attention to various family members trying to encourage them. She quotes various verses to them, hugs them, tells them God has a reason for all this, while empathizing with their struggles. Jim, an Eagle Scout and Deans list student in college, tearfully says that he has always tried to live his life right so that

they could avoid these kinds of things. Jana refuses to talk in session and just crosses her arms and sits. Mrs. Jones says that although Jana is very peaceful and sweet, she has found various household items smashed in her room and says she thinks Jana seems somewhat depressed since hearing the cancer diagnosis. John does not contribute much to the conversation, but says he loves his family and knows it will all work out somehow. John admits that he has always liked video games and often plays 2 hours a day but has been playing about 8-10 hours a day since the diagnosis. John says at one point, “maybe if I’d had enough faith this wouldn’t have happened, or maybe God would have already healed Momma.” Mrs. Jones corrects John gently and says his level of faith was not the cause of anything, and he listens but mentions that their aunt Jeanne told him “By His stripes we are healed” and that if they all had faith enough, there would be a supernatural healing. Mrs. Jones contradicts the aunt’s viewpoint, but John seems unsure of what to think and tearfully says he just wants to believe whatever will make his mom healthy again. All of the family members say that they chose you as their counselor because they want a Christian counseling approach to therapy as that is central to who they are, “not just some secular psychobabble”. They say they want at least weekly sessions, maybe some individual, some family, some marital...whatever you as the therapist thinks is best to help them best cope with this situation.